



Prepared for Humana — Medicare Advantage

May 2026

Confidential

Topics for discussion

01

Marley Medical overview

Who we are, the problem we solve, how our model works, and the clinical evidence we have built.

02

Working with Humana on Medicare Advantage

How Marley supports HCC capture & severity accuracy, moves HEDIS and Star measures, and reduces inpatient utilization in the cardiometabolic population.

03

Program design & contracting

Patient selection criteria, three contracting structures, evaluation design.



Welcome to Marley Medical

We are a virtual-first medical practice designed specifically for patients with cardiometabolic disease.

We manage hypertension, type 2 diabetes, lipid disorders and obesity with a team-based and integrated care model.

We currently bill fee-for-service, and we are actively building outcomes-based contracts with payer partners.



The cardiometabolic problem is large, expensive, and under-managed

Most patients with cardiometabolic disease are uncontrolled and high-cost.

60+%

of patients with cardiometabolic disease are uncontrolled

Source: O'Hearn et al. J Am Coll Cardiol 2022

23 %

of patients drive 71% of total healthcare spend

Source: AHRQ Multiple Chronic Conditions Chartbook

2/3

of cardiometabolic hospitalizations are avoidable

Source: AHRQ Potentially Avoidable Hospitalizations

\$600B

annual U.S. spend on CV and Endocrine disease

Source: KFF Health Costs / Disease-spend perspective

2–3 x

annual spend on cardiometabolic patients vs. others

Source: Gerteis et al. AHRQ

26+

average days to see a primary care provider

Source: Merritt Hawkins 2022 wait-time survey



Primary care can't carry this alone

A panel of 2,500 U.S. adults would need 26.7 hours per day for all guideline-recommended primary care.

26.7



hours per day

Primary care medicine — overwhelmed by guideline burden, chronic-disease complexity, and visit-volume pressure — is where most uncontrolled cardiometabolic patients live today. The result is the 60%+ uncontrolled rate on the previous slide.

Journal of General Internal Medicine

Revisiting the Time Needed to Provide Adult Primary Care

Porter J, Boyd C, Skandari MR, et al.

"Primary care physicians would need 26.7 hours per day to provide all recommended preventive, chronic disease, and acute care to a panel of 2,500 U.S. adults. Without restructuring, the guidelines simply cannot be delivered."

J Gen Intern Med 38(1) — 2022



A new category of care is needed

Complex cardiometabolic patients fall between specialists and PCPs. Marley is built for the gap.

Specialists

High clinical expertise. Paid per procedure. Limited capacity. Don't manage the whole patient.



Marley Medical

Comprehensive & integrated care for chronic disease – paid for outcomes.

PCPs

General population. Paid for time-based visits. Overburdened – 26.7 hrs/day to meet guidelines.

Why this gap matters

- Specialists are scarce and procedure-paid – wrong economics for chronic disease.
- PCPs are time-paid and capacity-constrained – wrong economics for high-touch care.
- Marley is positioned to manage the whole cardiometabolic patient with outcomes-based economics.



A multidisciplinary care team – under one roof

Specialist expertise plus continuous engagement. Every patient, every condition.



How it works

- One unified medical record, shared across the team.
- Continuous data from home BP cuff, weight, and labs.
- Asynchronous messaging + scheduled video visits.
- Coordinated escalation to in-person care when needed.



Comprehensive scope, complex panel

What we manage, and who we already serve.



Conditions we manage

- **Hypertension**
All stages, including Stage 2 and crisis.
- **Type 2 diabetes**
Including patients on basal insulin and GLP-1 RAs.
- **Pre-diabetes**
Risk-adjusted lifestyle and pharmacologic intervention.
- **Hyperlipidemia**
Statin-first; ezetimibe and PCSK9i where indicated.
- **Obesity / overweight**
Behavioral + pharmacotherapy with GLP-1 agents.



Our current panel profile

- **Mean age 56 / max 90**
Cardiometabolic skews older — many already MA-eligible.
- **3.3 chronic conditions**
Mean / 3 median. Polychronic by design.
- **34% mental health dx**
Anxiety, depression are common comorbidities.
- **62% diagnosed obesity**
Weight is central to the cardiometabolic risk model.
- **84% have existing PCP**
We complement primary care; we don't replace it.



The 20–30% not well served by today's options

We sit between specialists, care managers, and PCPs – and we get paid for outcomes.

Specialists

→ Paid for procedures



Marley Medical

→ **Paid for outcomes**

Care managers

→ Paid per attribute / call

PCPs

→ Paid for time-based visits

We currently bill fee-for-service. We are actively building outcomes-based contracts – capitated and milestone-based – with payer partners.



Three ways patients come to Marley

Direct, PCP referral, and payer identification – all converge on the same care.

01



Patient self-refers

Direct response from our brand and marketing. The patient finds us, decides to join, and we onboard them.

Largest channel today.

02



PCP refers

A primary care physician identifies a patient who would benefit from Marley and refers them to us. We coordinate care back to the PCP.

Growing channel.

03



Payer identifies

Payer uses claims data to identify members who fit Marley's profile and unengaged in care. We outreach with the payer's endorsement.

Highest-yield for MA.

We refer up for imaging, procedures, and specialty care when patient needs go beyond our cardiometabolic scope.



The Marley patient journey

From lead to controlled disease – averaging 2+ touch points a week.



Lead

Direct response from
brand & marketing.



Video visit

First MD visit
in under one week.



Ship kit

Cellular BP cuff + meds
to the patient's door.



Continuous monitoring

Cellular readings
flow to the team.



Async messaging

Care team responds,
medications titrated.



Follow-up + lifestyle

Health coach + dietitian
close the loop.



Patients stay engaged – week after week

Engagement is the operating system of outcomes. Here's how ours runs.

78%

of patients active in last 30 days

<1 wk

to first MD video visit

75%

retention at 1 year

5

device readings per patient per week

2.5

messages read per patient per week

86

NPS score, last 30 days

We've done this before – at scale, with payers

Our team built and validated data-driven chronic disease care at Propeller Health.

Marley's founders and roughly a third of our team came from Propeller Health, where we built and ran connected respiratory disease management for asthma and COPD.

Propeller partnered with national MA, Medicaid, and commercial payers, including multi-year programs measured on ED and hospitalization reduction.

We are applying the same data-driven playbook to the cardiometabolic population – a larger, costlier, and even more under-managed segment.

57%

reduction in ED + hospitalizations
(Asthma)

33%

reduction in ED + hospitalizations (COPD)

Propeller Health peer-reviewed evidence – Marley founder & ~1/3 of team prior employer.

JAMA Internal Medicine

Sensor-driven self-management for adults with poorly controlled asthma

Merchant R, et al. – multi-year RCT

Reduced rescue-inhaler use by 78% and significantly improved control across the cohort.

Annals of the American Thoracic Society

Population-scale digital therapeutic in COPD

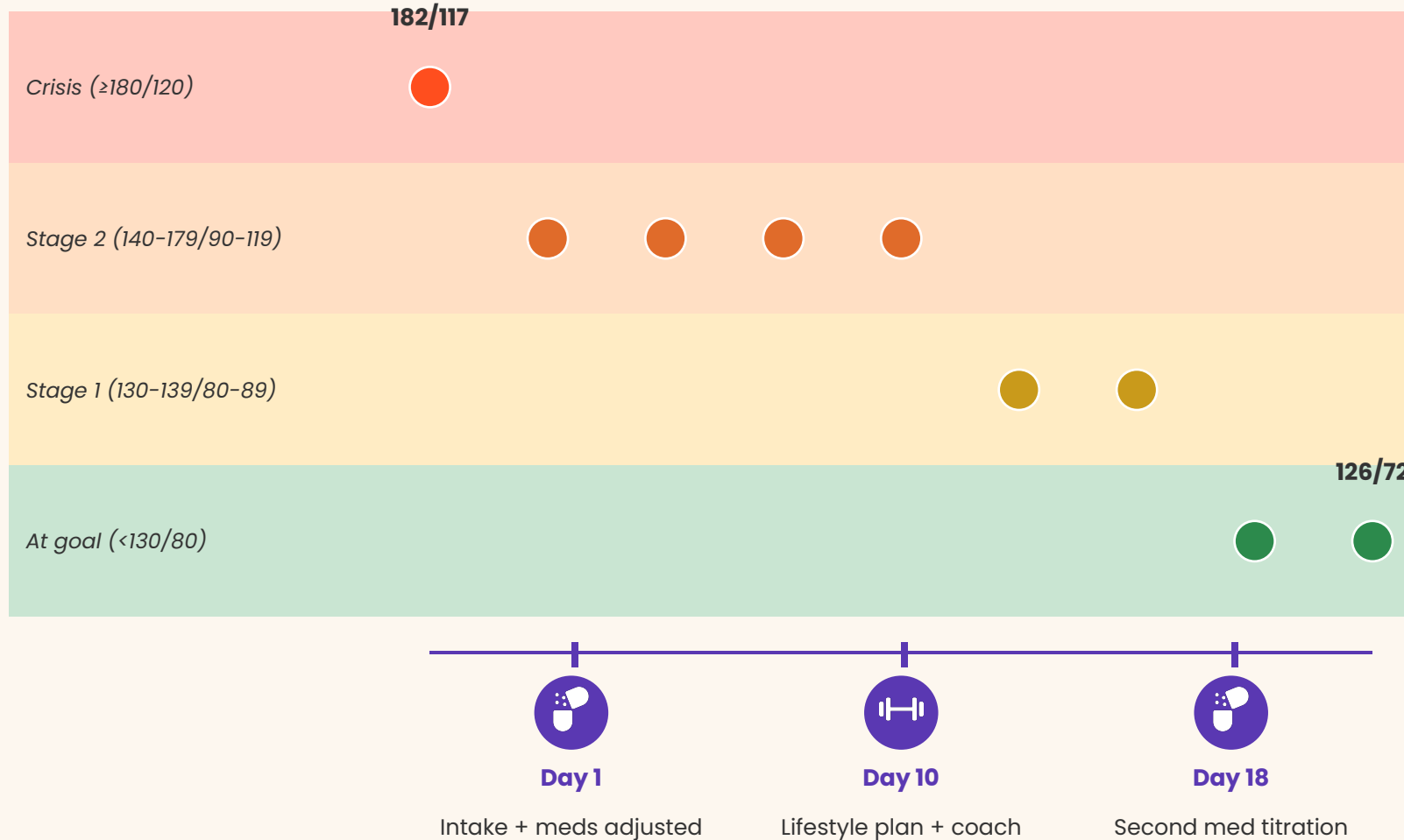
Barrett M, et al.

Reduced healthcare utilization in patients with COPD using connected inhaler technology.



From hypertensive crisis to controlled – in 23 days

A representative recent patient case. Two medication changes, lifestyle support, near-daily engagement.



Within 23 days

BP dropped from 182/117 to 126/72.

Two medication titrations led by our pharmacist and physician.

Daily home BP readings via cellular cuff.

Health coach lifestyle plan — sodium, sleep, stress.

Asynchronous messaging averaging 4–5 touchpoints per week.



Clinical results: patients reach goal and stay there

Marley-direct outcomes data, plus the downstream risk reductions the literature predicts.

Our hypertension cohort — March 2024

70%

at guideline clinical goal at 6 months

14.8 mmHg

average BP reduction at 12 weeks (Stage 2 entry)



Confirmed in our
1,250-patient panel

across 6 states

Downstream risk reduction the literature predicts

For every 10 mmHg reduction in systolic BP (Ettehad et al., Lancet 2015, n=614,000):

20%

less major CV events

27%

less stroke

28%

less CHF

17%

less CHD

13%

less all-cause mortality



How Marley works with Humana

Three Humana priorities – accurate HCC capture, HEDIS and Star measure performance, and inpatient utilization reduction – sit inside the natural workflow of our cardiometabolic care model.



Humana's 2026 moment

This isn't a generic MA pitch. Here's the specific pressure Marley is built to relieve.

~20%

**of members in 4+ Star plans
for 2026**

Down from ~25% the prior year.
Quality bonus revenue is at stake.

~\$3.5B

**revenue headwind cited for
the Stars decline**

Star Ratings flow straight to MA
rebate dollars and benefit
richness.

V28

**risk model is deflating risk
scores**

Cardiometabolic codes are
squarely in the re-weighting.
Accurate capture matters more,
not less.

~1M

**new-to-plan members
from AEP growth**

First-year members are the
hardest to code accurately —
exactly where Marley's intake
shines.

The fastest paths back to top-quartile Stars run through accurate risk capture, moved HEDIS measures, and fewer avoidable admissions — in the polychronic members who drive both.

Marley is designed for Humana's MA economics

Our model choices align with your priorities – by design, not by retrofit.



What Marley is designed for

- A focused, expensive patient population – cardiometabolic polychronics, average age 56 with 3.3 chronic conditions.
- Comprehensive care model with high engagement – 2+ touch points per week, 78% active monthly, 75% annual retention.
- A care surface broad enough to control a meaningful share of medical spend – HTN, T2D, lipids, obesity.
- Selective referral to local in-person care – coordinated, not duplicative.
- Fee structures we will put at risk on outcomes – up to 30% of fees against quality.



What Humana needs to move

HCC capture & severity

Accurate documentation of cardiometabolic HCCs – especially in new-to-plan members – under V28.

HEDIS & Star Ratings

Direct measure performance: CBP, HBD, SUPD, SPC, MRP, TRC. Stars revenue impact is material.

Inpatient utilization

Polychronic patients drive avoidable hospitalizations. Better control = lower MLR.

Member engagement

Up to 30% of MA membership is unengaged. We bring them back.



Marley sits beside CenterWell – not against it

We complement Humana's owned primary care, we don't compete with it.

CenterWell / owned & aligned primary care

Owns the attributed primary-care relationship. Strong at whole-person primary care, preventive visits, and the broad MA panel. Capacity is the constraint – a PCP panel can't deliver high-touch specialty cardiometabolic management to its most complex tail.



Marley / the polychronic cardiometabolic tail

Takes the uncontrolled, unengaged, polychronic cardiometabolic members a PCP panel can't serve at depth. Refers back to the PCP for everything outside our scope. One shared record, coordinated – not duplicative.

Why this works for Humana

● No cannibalization

Marley targets members CenterWell isn't actively managing at depth – the unengaged, uncontrolled tail.

● Protects attribution

Selection explicitly screens out members in a conflicting CenterWell or capitated PCP arrangement.

● Compounds owned-asset value

Better control and capture in the complex tail lifts the same Stars and RAF metrics CenterWell is measured on.



We know how to partner with payers – and deliver

Our team has spent over a decade building outcomes-based programs with national payers.

Marley founders and roughly a third of our team came from Propeller Health, where we ran multi-year programs with national MA, Medicaid, and commercial payers – measured on ED and hospitalization reduction in chronic disease.

We carried that operating model into Marley:

- Co-built clinical protocols with payer medical leadership.
- Quarterly outcomes reporting against agreed measure sets.
- Fees-at-risk on quality – not just on enrollment.
- Compliant claims, clean documentation, ICD-10 detail.

13

active Midwest payer contracts

31M

covered lives

77%

payer coverage in active markets

Backed by

a16z

CRV

**Rock
Health**

Comparable: Ochsner Digital Medicine Program

\$204 PMPM

in HTN cost savings

\$163 PMPM

in T2DM cost savings

Published methodology, peer-reviewed program design.



Where Marley creates value for Humana

In priority order. The next three slides go deeper on the first three.

01 HCC capture & severity



Cardiometabolic intake, continuous monitoring, and a multidisciplinary team produce comprehensive, well-documented diagnoses. Directional 0.2–0.4 added RAF per engaged member — detailed next slide.

PRIMARY

02 HEDIS & Star measures



Direct alignment with CBP, HBD, SUPD, SPC, MRP, TRC, and PCR. Our care model moves these measures by design.

03 Inpatient utilization



Frequent engagement and proactive titration prevent the cardiometabolic events that drive avoidable admissions.

04 Member engagement



We re-engage the 'unengaged' — 78% of our patients active monthly, 75% retained at 1 year, NPS 86.



Built for accurate HCC capture

Our intake and care model surface cardiometabolic diagnoses and severity that fee-for-service primary care misses.



Comprehensive intake + labs

Every patient gets a full cardiometabolic workup at intake: HbA1c, fasting glucose, lipid panel, eGFR, urinalysis, microalbumin.

Surfaces CKD (eGFR <60), diabetes severity by A1c band, and dyslipidemia coding routinely missed in PCP visits.



Continuous monitoring

Cellular BP cuff readings stream into the chart five times per week. Weight and BMI tracked at every encounter.

We see hypertensive crisis vs. Stage 2 vs. Stage 1 in the data — not just at the annual visit. BMI confirms obesity HCC severity.



Specialist diagnostic eye

Cardiology, endocrinology, internal medicine, and pharmacist all on the same chart. Standard protocols cover the full cardiometabolic risk model.

Hypertensive heart disease, hypertensive CKD, T2DM with complications, PVD — diagnoses that need a specialist's eye don't get missed.



Documentation by design

Structured intake protocol, Canvas EMR, and 2+ encounters per week create many more opportunities for accurate documentation.

M.E.A.T. criteria — monitor, evaluate, assess, treat — are explicit in every encounter note. Clean ICD-10 detail in every claim.

0.2–0.4

directional added RAF per engaged member

Illustrative for a polychronic cardiometabolic cohort, not a guarantee. Realized lift depends on member engagement and Humana's coding workflow — we'll set specific capture-rate and accuracy targets jointly in program design.



HEDIS and Star measures Marley moves

The measures that matter most for Stars in the chronic disease population.

Direct hits – Marley is the workflow

CBP

Controlling High Blood Pressure

70% of our HTN patients reach goal at 6 months.

HBD

HbA1c Control for Diabetes

Core scope; pharmacist-led titration.

SUPD

Statin Use in Diabetes

Pharmacist med rec; statin initiation per guideline.

SPC

Statin for Cardiovascular Disease

Protocolized initiation and adherence support.

KED / EED

Kidney & Eye Exams for Diabetes

Labs and referrals coordinated by the care team.

Indirect – Marley supports the workflow

MRP

Medication Reconciliation

Built into the pharmacist workflow every time.

TRC

Transitions of Care

Clinical escalation + active care-team contact.

PCR

Plan All-Cause Readmissions

Lower via tighter control in the polychronic cohort.

FMC

Follow-up after ED Visit

We are the post-ED follow-up touchpoint.

CAHPS

Member experience

Stars-weighted. NPS 86 speaks to this.



Inpatient reduction: mechanism, math, and comparables

Better cardiometabolic control compresses the events that drive avoidable hospitalizations.

The mechanism

- 2+ touch points per week catches deterioration before it becomes an event.
- Pharmacist-led titration drives 70% to BP goal — 10 mmHg tracks to 28% less CHF.
- Continuous monitoring triggers proactive outreach, not reactive ED visits.

Comparable evidence – same team, same playbook

57%

fewer ED + hospitalizations (Asthma, Propeller)

33%

fewer ED + hospitalizations (COPD, Propeller)

What avoidable events cost

First-quarter Medicare costs of acute CV events:

\$39k

Percutaneous coronary intervention (PCI)

\$77.7k

Coronary artery bypass graft (CABG)

\$37k

Stroke (first quarter)

AHRQ: 2 of 3 cardiometabolic hospitalizations are avoidable.

Easy to work with – by design

Six attributes that make Marley fit cleanly into Humana's contracting model.



Broad population

Not just one disease or the highly motivated
— we serve the polychronic 5%.



Guideline-based

Evidence-based protocols built into the
platform. Care decisions auditable
end-to-end.



Highly scalable

Tech-enabled team scales by adding
licensed providers — no real estate, no
panel ceilings.



Active payer contracts

Currently contracted with 13 top Midwest
payers across 6 states; ready to extend.



Value-based ready

We will put fees at risk on quality — up to
30%. Capitation and milestones, not just FFS.



Path to geographic expansion

Clinicians licensed in 26 states beyond our
current 6. Payer partners accelerate.



Three stages of program design

From who we serve, to how we get paid, to how we know it worked.

01

Patient selection

We work together to identify members who fit Marley's profile and would benefit most.

- ICD-10 group inclusion / exclusion criteria
- Not attributed to a VBC / risk arrangement
- Unengaged in care over last 12-18 months
- Failing management — recent ED or admission

02

Contracting structure

Three flexible structures — match Humana's preferred economics.

- FFS + quality bonus (+/- 30%)
- Capitated PMPM rate with fees at risk
- Milestone-based — enrollment, activity, outcomes
- Hybrids welcomed; we are flexible by design

03

Evaluation design

We co-select the metrics and the windows. Joint reporting, quarterly.

- HCC capture & risk-score lift (with codes named)
- HEDIS / Star measures — CBP, HBD, SUPD, others
- Inpatient and ED admit-rate reduction
- Member experience — CAHPS, NPS



Patient selection criteria for Humana MA

Four criteria, tuned to Humana MA. We refine these jointly during program design.

01 Cardiometabolic clinical profile



Member has uncontrolled HTN, T2D, lipid disorder, or BMI ≥ 30 . We co-define the exact ICD-10 inclusion/exclusion list.

02 Not attributed in another risk arrangement



Member is not attributed to a Humana CenterWell, VBC, or capitated PCP relationship that would conflict with Marley's role.

03 Unengaged or under-engaged in care



No PCP claim in last 12–18 months OR no cardiometabolic Rx fills in last 6 months. Up to 30% of MA membership sits here.

04 Recent failure of management



2+ ED visits in last 12 months OR 1+ inpatient admission.
Highest-priority members for Marley intervention.



Three contracting structures – pick what fits

Each structure puts Marley's economics on the line for outcomes. We're flexible – all three are operationally proven.

01 FFS + Quality Bonus



Standard fee-for-service claims, plus +/- 30% quality bonus at 12 months.

Pros	Familiar to operations. Low integration lift.
Cons	Marley revenue volatile if utilization shifts.
At risk	30% of fees, evaluated annually.
Best for	First contract with a payer; pilot phase.

02 Capitation + Fees at Risk



Monthly PMPM rate on active patients, with up to 30% at risk against quality measures.

Pros	Predictable for both sides. Aligns incentives.
Cons	Requires clear active-patient definition.
At risk	Up to 30% of monthly PMPM.
Best for	Steady-state programs with reliable enrollment.

03 Milestone-Based



Claims submitted based on agreed clinical and engagement milestones – not time.

Pros	Cleanest outcomes-economics alignment.
Cons	Requires sophisticated tracking infrastructure.
At risk	All fees against milestone achievement.
Best for	Mature payer partnerships, Stars-leveraged programs.



A concrete place to start

One market, one focused cohort, a clean 12-month read. Here's what we'd propose.

Who

~2,500 members

Unengaged, uncontrolled, polychronic cardiometabolic MA members in one Humana market where Marley is licensed.

Structure

FFS + quality

Start at the low-integration end — up to 30% of fees at risk on jointly-agreed quality. Step toward capitation as the data matures.

Measures

HCC · HEDIS · IP

Risk-capture accuracy and RAF lift, the named HEDIS/Star measures, and inpatient + ED admit-rate — baselined against a matched cohort.

Timeline

12 months

Quarterly joint reporting. Pre-agreed go / grow / stop thresholds at month 12 before any expansion.

Small enough to stand up fast. Structured enough to read cleanly. Designed to earn the expansion.

Thank you

Let's design the program together.

Chris Hogg, CEO

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marley
medical

Clinical & economic data deep dive

Reference numbers behind the program-design conversation.



Comparable evidence: Ochsner Digital Medicine

Peer-reviewed digital chronic disease program with published economic outcomes.

\$204

PMPM cost savings in hypertension

\$163

PMPM cost savings in type 2 diabetes

What this tells us – and where it doesn't apply directly

Ochsner ran a connected, team-based chronic-disease program in a defined population and measured per-member-per-month savings against a control. Marley's care model is directly analogous; our population is older, more polychronic, and more virtual-native. We expect comparable savings — measured jointly with Humana — once enrolled at scale.



What uncontrolled cardiometabolic disease costs

Acute event costs – the dollar amounts on the other side of better control.

\$39,000

First-quarter Medicare cost of PCI

Percutaneous coronary intervention

\$77,701

First-quarter Medicare cost of CABG

Coronary artery bypass graft

\$37,000

First-quarter Medicare cost of stroke

Ischemic stroke admission + post-acute

The math is simple.

Avoiding even one acute CV event per 100 members pays for an annual cardiometabolic program at population scale.